



Division of Provider Services
& Quality Assurance
P.O. Box 8059, Slot S404
Little Rock, AR 72203-8059

August 7, 2025

Charlotte Lockhart, Administrator
Woodridge Of Forrest City, Llc
1521 Albert St
Forrest City, AR 72335

Dear Ms. Lockhart:

During the **Complaint Investigation Survey** conducted on **July 14, 2025**, your facility was found to be in compliance with program requirements. **Please email the signed CMS 2567**
Breanna.marengo@dhs.arkansas.gov.

If you have any questions, please contact your reviewer: **Breanna Marengo at 501-320-6280 or email to Breanna.Marengo@dhs.arkansas.gov.**

Sincerely,

A handwritten signature in black ink, appearing to read "B. Marengo". The signature is written in a cursive, flowing style.

for Jeff Rosenbaum

Breanna Marengo, RN QA Reviewer
DPSQA/Office of Long Term Care
Survey and Certification Section

bbm

DEPARTMENT OF HEALTH AND HUMAN SERVICES
CENTERS FOR MEDICARE & MEDICAID SERVICES

Approved 08/07/2025 BBM

PRINTED: 07/28/2025
FORM APPROVED
OMB NO. 0938-0391

STATEMENT OF DEFICIENCIES AND PLAN OF CORRECTION		(X1) PROVIDER/SUPPLIER/CLIA IDENTIFICATION NUMBER: 04L115	(X2) MULTIPLE CONSTRUCTION A. BUILDING _____ B. WING _____		(X3) DATE SURVEY COMPLETED C 07/14/2025
NAME OF PROVIDER OR SUPPLIER WOODRIDGE OF FORREST CITY, LLC			STREET ADDRESS, CITY, STATE, ZIP CODE 1521 ALBERT ST FORREST CITY, AR 72335		
(X4) ID PREFIX TAG	SUMMARY STATEMENT OF DEFICIENCIES (EACH DEFICIENCY MUST BE PRECEDED BY FULL REGULATORY OR LSC IDENTIFYING INFORMATION)	ID PREFIX TAG	PROVIDER'S PLAN OF CORRECTION (EACH CORRECTIVE ACTION SHOULD BE CROSS-REFERENCED TO THE APPROPRIATE DEFICIENCY)	(X5) COMPLETION DATE	
N 000	Initial Comments Note: The CMS-2567 (Statement of Deficiencies) is an official, legal document. All information must remain unchanged except for entering the plan of correction, correction dates, and the signature space. Any discrepancy in the original deficiency citation(s) will be reported to the Dallas Regional Office (RO) for referral to the Office of the Inspector General (OIG) for possible fraud. If information is inadvertently changed by the provider/supplier, the State Survey Agency (SA) should be notified immediately. Census 44. AR00039411 was in compliance.	N 000	The Director of Nursing will be responsible for correcting the deficiency and implementing the acceptable plan of correction. The policy has been updated to define "provider" as the licensed practitioner who initiated the restraint order, which may include a Physician, Nurse Practitioner (NP), or Physician Assistant (PA). These individuals must be notified of the face-to-face assessment results as outlined in the updated training materials. The order/assessment authentication will serve as notification of event and Face-to-face assessments. A revised nursing documentation template now includes required fields for: Time of the face-to-face assessment, Resident's physical and psychological status, Resident's behavior, Appropriateness of the intervention, Any complications observed. A notification protocol has been implemented to ensure that the ordering provider (MD, NP, or PA) is informed of the face-to-face results in alignment with CMS expectations. Order authentication will serve as notification of event.		
N 145	ORDERS FOR USE OF RESTRAINT OR SECLUSION CFR(s): 483.358(f) Within 1 hour of the initiation of the emergency safety intervention a physician, or other licensed practitioner trained in the use of emergency safety interventions and permitted by the state and the facility to assess the physical and psychological wellbeing of residents, must conduct a face-to-face assessment of the physical and psychological wellbeing of the resident, including but not limited to- (1) The resident's physical and psychological status; (2) The resident's behavior; (3) The appropriateness of the intervention	N 145	The Director of Nursing will train all licensed medical and nursing staff on the documentation process and notification protocol by August 16, 2025. The training will emphasize provider roles, expectations for timely documentation, and regulatory compliance. All staff will sign an attestation upon completion of the training. Staff will not be allowed back on the floor if training is not completed by the due date and training must be completed before returning to work their shift. Monitoring: Audits within the next business day will be conducted by the Director of Nursing and Director of Quality and Risk Management to ensure the 1-hour face-to-face assessment is: Completed by a qualified staff member, Documented accurately with time and required content, Followed by timely notification to the ordering provider as required. Audit findings will be reviewed monthly with the Executive Leadership Team during the Committee of Whole/Quality committee, and non-compliance will be addressed with retraining or corrective action. Monitoring will be ongoing until 100% compliance for three consecutive months. Following, monitoring/audits will occur bi-monthly. Completion Date: August 16, 2025		

LABORATORY DIRECTOR'S OR PROVIDER/SUPPLIER REPRESENTATIVE'S SIGNATURE

TITLE

(X6) DATE

 Director of Quality and Risk Management 08/07/2025

Any deficiency statement ending with an asterisk (*) denotes a deficiency which the institution may be excused from correcting providing it is determined that other safeguards provide sufficient protection to the patients. (See instructions.) Except for nursing homes, the findings stated above are disclosable 90 days following the date of survey whether or not a plan of correction is provided. For nursing homes, the above findings and plans of correction are disclosable 14 days following the date these documents are made available to the facility. If deficiencies are cited, an approved plan of correction is requisite to continued program participation.

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N 145	Continued From page 1 measures; and (4) Any complications resulting from the intervention. This ELEMENT is not met as evidenced by: Based on record review and interview, the facility failed to ensure a face-to-face assessment was conducted by a physician, or other licensed practitioner, within an hour of initiation of a restraint of one (Resident #5) of six residents reviewed. The findings include: Review of Resident #5 ' s Master Treatment Plan dated 05/23/25, indicated Resident #5 had diagnoses which included major depressive disorder, generalized anxiety disorder, post-traumatic stress disorder, and attention-deficit hyperactivity disorder. Review of an Emergency Safety Intervention Justification Packet completed and signed by the nurse on 05/23/25 at 12:35 PM and signed by the physician on 05/23/25 at 12:40 PM, indicated Resident #5 was in a chemical restraint. Review of the Seclusion/Restraint/Emergency Medication One Hour Face to Face Medical and Behavioral Evaluation indicated the date and time of debriefing was 05/23/25 at 12:35 PM. The Chemical Restraint ended at 12:43 PM. Review of the One Hour Face to Face Medical and Behavioral Evaluation did not indicate that the face-to-face evaluation was completed within one hour of the physical restraint, as it had been signed on 05/23/25 at 12:35 PM, the same time	N 145			

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N 145	Continued From page 2 the restraint had been initiated. During an interview on 07/09/25 at 9:04 AM, the Director of Nursing reported that the time and date of a face-to-face evaluation must be documented within an hour, not at the time the restraint had been initiated. The policy titled " Emergency Safety Interventions " dated 01/2025, read in part, "a face-to-face assessment must be completed by the Licensed Independent Practitioner Designee or a registered nurse that has been clinically qualified or trained to perform this assessment no greater [than] 60 minutes from the initiation of the restraint or seclusion."	N 145	The Director of Nursing, will be responsible for correcting the deficiency and implementing the acceptable plan of correction. To correct this deficiency: All staff will receive retraining on the importance, timing, and documentation of debriefs following any physical restraint or hold. Nursing staff will be re-trained to return and attempt a follow-up debrief the next day if the resident initially refuses, ensuring that all reasonable efforts are made to engage the resident. If the resident refuses three times within 24 hours, the nursing department will notify the resident's guardian of the refusal and document the efforts made. The Handle With Care instructor will conduct mandatory retraining sessions by August 16, 2025, which will cover: De-escalation strategies and trauma-informed debriefing, The requirement to complete a staff and resident debrief within 24 hours of an intervention, The procedure for repeated refusals and guardian notification. Attestations will be signed by all staff to verify completion and understanding of the training. Staff will not be allowed back on the floor if training is not completed by the due date and training must be completed before returning to work their shift. Monitoring: The Safety Team, in collaboration with the Director of Nursing and Director of Quality and Risk Management, will conduct audits (within one business day) of restraint episodes to confirm: Completion and documentation of staff and resident debriefs, Re-attempts for residents who initially refuse, Guardian notification when applicable. Audit findings will be reviewed monthly with the Executive Leadership Team during the Committee of Whole/Quality Meeting, and non-compliance will be addressed with retraining or corrective action. Monitoring will be ongoing until 100% compliance for three consecutive months. Following, monitoring/audits will occur bi-monthly. Completion Date August 16, 2025		
N 179	NOTIFICATION OF PARENT(S) OR LEGAL GUARDIAN CFR(s): 483.366(b) [If the resident is a minor as defined in this subpart] the facility must document in the resident's record that the parent(s) or legal guardian(s) has been notified of the emergency safety intervention, including the date and time of notification and the name of the staff person providing the notification. This ELEMENT is not met as evidenced by: Based on record review and interview, the facility failed to ensure that documentation for guardian notification was in a resident's clinical record after the implementation of an emergency safety intervention for one (Resident #5) of six residents reviewed for restraint documentation. The findings include:	N 179			

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N 179	Continued From page 3 Review of Resident #5 ' s Master Treatment Plan dated 05/23/25, indicated Resident #5 had diagnoses which included major depressive disorder, generalized anxiety disorder, post-traumatic stress disorder, and attention-deficit hyperactivity disorder. Review of an Emergency Intervention packet, completed and signed by the nurse on 05/23/25 at 12:43 PM and signed by the physician on 05/23/25 at 12:35 PM, indicated Resident #5 was in a chemical restraint. Review of an Emergency Safety Intervention Justification packet dated 05/23/25, indicated a physical restraint was conducted at 12:35 PM, due to a "physical altercation with a peer, physical aggression with staff, " and being " unable to self-regulate." A chemical restraint was administered at 12:43 PM, due to "physical aggression toward staff and verbal threats toward staff and peers." The attempts made to contact the guardian for the restraint section was not completed. The policy titled, " Emergency Safety Interventions (date 01/2025, read in part, " A Healthcare nurse must notify the parent(s) or legal guardian(s) whenever a resident requires an emergency safety intervention as soon as possible after the initiation of the emergency safety intervention, but within 24 hours. Documentation occurs in the resident's medical record of such notification and the date, time, and name of nurse providing the notification. "	N 179			
N 188	POST INTERVENTION DEBRIEFINGS CFR(s): 483.370(a)	N 188			

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N 188	Continued From page 4 Within 24 hours after the use of the restraint or seclusion, staff involved in an emergency safety intervention and the resident must have a face-to-face discussion. This discussion must include all staff involved in the intervention except when the presence of a particular staff person may jeopardize the wellbeing of the resident. Other staff and the resident's parent(s) or legal guardian(s) may participate in the discussion when it is deemed appropriate by the facility. The facility must conduct such discussion in a language that is understood by the resident and by the resident's parent(s) or legal guardian(s). The discussion must provide both the resident and staff the opportunity to discuss the circumstances resulting in the use of restraint or seclusion and strategies to be used by the staff, the resident, or others that could prevent the future use of restraint or seclusion. This STANDARD is not met as evidenced by: Based on record review and interview, the facility failed to ensure a timely and effective face to face debriefing was completed within 24 hours for residents following a restraint intervention for three (Resident #1, Resident #2, and Resident #5) of six residents reviewed. The findings include: Resident #1 Review of Resident #1 's Master Treatment Plan dated 04/17/25, indicated Resident #1 had diagnoses which included manic depression which was recurrent and severe without psychosis, general anxiety disorder, and	N 188	The Director of Nursing, will be responsible for correcting the deficiency and implementing the acceptable plan of correction. To correct this deficiency: All staff will receive retraining on the importance, timing, and documentation of debriefs following any physical restraint or hold. Nursing staff will be re-trained to return and attempt a follow-up debrief the next day if the resident initially refuses, ensuring that all reasonable efforts are made to engage the resident. If the resident refuses three times within 24 hours, the nursing department will notify the resident's guardian of the refusal and document the efforts made. The Handle With Care instructor will conduct mandatory retraining sessions by August 16, 2025, which will cover: De-escalation strategies and trauma-informed debriefing, The requirement to complete a staff and resident debrief within 24 hours of an intervention, The procedure for repeated refusals and guardian notification. Attestations will be signed by all staff to verify completion and understanding of the training. Monitoring: The Safety Team, in collaboration with the Director of Nursing and Director of Quality and Risk Management, will conduct audits (within one business day) of restraint episodes to confirm: Completion and documentation of staff and resident debriefs, Re-attempts for residents who initially refuse, Guardian notification when applicable. Audit findings will be reviewed monthly with the Executive Leadership Team during the Committee of Whole/Quality meeting, and non-compliance will be addressed with retraining or corrective action. Completion Date: August 16, 2025		

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N 188	Continued From page 5 post-traumatic stress disorder. Review of a Nurse Progress note, signed by the nurse on 04/17/25, indicated Resident #1 was placed in a physical (escort) restraint from 8:12 PM to 8:13 PM. Review of Resident #1 ' s Emergency Safety Intervention (ESI) packet dated 04/17/25, signed by the nurse at 8:15 PM, and signed by the physician on 04/17/25 at 8:20 PM, indicated a physical (escort) restraint was conducted at 8:12 PM, due to Resident #1 being out of area. Review of Resident #1 ' s Seclusion/Restraint/ Emergency Medication One Hour Face to Face Medical & Behavioral Evaluation did not indicate that a face-to-face evaluation was completed within one hour of the physical escort. The documented time was the time of the physical escorted restraint; there was no documented time of a one hour face to face discussion within 24 hours of the physical escort. Resident #2 Review of Resident #2's Master Treatment plan dated 05/16/25, revealed the resident had diagnoses which included major depressive disorder, which was recurrent and severe without psychosis, generalized anxiety disorder, disruptive mood dysregulation disorder, and attention deficit hyperactivity disorder. Review of an Emergency Safety Intervention Justification Packet signed by the nurse on 07/07/25, indicated Resident #2 had to be placed in a physical restraint from 7:00 AM to 7:02 AM.	N 188			

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N 188	Continued From page 6 Review of an Emergency Intervention packet, completed and signed by the nurse on 07/07/25 at 7:02 PM, and signed by the physician on 07/07/25 at 9:15 PM, indicated Resident #2 was placed in a physical restraint on 07/07/25 at 7:00 AM to 7:02 AM. Review of the post intervention debriefing documentation, included with the safety intervention documentation dated 07/07/2025, did not contain any information for the staff face to face discussion. Resident #5 Review of Resident #5 ' s Master Treatment Plan dated 05/23/25, revealed the resident had diagnosis which included major depressive disorder, generalized anxiety disorder, post-traumatic stress disorder, and attention-deficit hyperactivity disorder. Review of an Emergency Intervention packet, completed and signed by the nurse on 05/23/25 at 12:43 AM and signed by the physician on 05/23/25 at 12:45 AM revealed Resident #5 was placed in a chemical restraint on 05/23/25 at 12:35 PM, due to "physical aggression towards staff and verbal threats towards staff and peers." Review of the post intervention debriefing documentation page, included with the safety intervention documentation dated 05/23/25, did not contain any information in the client debriefing section, and there was missing information on the staff debriefing section, which included the date, time, and signatures of staff. A Patient Emergency Intervention Debriefing form had three attempts documented at the top; however, it	N 188			

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N 188	Continued From page 7 was not indicated that the client refused the debriefing, nor who attempted the debriefing. During an interview on 07/09/25 at 9:04 AM, the Director of Nursing (DON) reported that the time and date of the face-to-face evaluation and discussion must be documented. In addition, the DON acknowledged that after a resident was released from the restraint, a nurse was to attempt a debriefing with the resident, the information section should be completed, and/or the reason for the requirements not being completed. The facility policy titled, " Emergency Safety Interventions " dated 01/2025, read in part, " the resident and a licensed nurse, will meet in a post-intervention conference to discuss the event and obtain feedback concerning precipitating events and what could have been done to prevent the event. This must take place within 24 hours of the restraint. The resident will then complete a debriefing form, and it will be placed in the resident ' s chart behind the Restraint Justification form."	N 188			
N 189	POST INTERVENTION DEBRIEFINGS CFR(s): 483.370(b) Within 24 hours after the use of restraint or seclusion, all staff involved in the emergency safety intervention, and appropriate supervisory and administrative staff, must conduct a debriefing session that includes, at a minimum, a review and discussion of - 483.370(b)(1) The emergency safety situation that required the intervention, including discussion of the precipitating factors that led up	N 189			

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N 189	Continued From page 8 to the intervention; This ELEMENT is not met as evidenced by: Based on record review and interview, the facility failed to ensure a timely and effective staff debriefing was provided or documented within 24 hours after two (Resident #2 and Resident #5) of six residents were placed in a restraint. The findings include: Resident #2 Review of Resident #2's Master Treatment Plan dated 05/16/25, indicated Resident #2 had diagnoses which included major depressive disorder, which was recurrent and severe without psychosis, generalized anxiety disorder, disruptive mood dysregulation disorder, and attention deficit hyperactivity disorder. Review of a Physician Order for Precautions, signed by the nurse on 07/07/25, indicated Resident #2 had to be placed in a physical restraint from 7:00 AM to 7:02 AM. Review of an Emergency Intervention packet, completed and signed by the nurse on 07/07/25 at 7:02 PM, and signed by the physician on 07/07/25 at 9:15 PM, indicated Resident #2 was placed in a physical restraint on 07/07/25 at 7:00 AM to 7:02 AM. Review of the post intervention debriefing documentation page, included with the safety intervention documentation dated 07/07/2025, did not contain any information on the date of the debriefing or the staff present.	N 189	The Director of Nursing, will be responsible for correcting the deficiency and implementing the acceptable plan of correction. To correct this deficiency: All staff will receive retraining on the importance, timing, and documentation of debriefs following any physical restraint or hold. Nursing staff will be re-trained to return and attempt a follow-up debrief the next day if the resident initially refuses, ensuring that all reasonable efforts are made to engage the resident. If the resident refuses three times within 24 hours, the nursing department will notify the resident's guardian of the refusal and document the efforts made. The Handle With Care instructor will conduct mandatory retraining sessions by August 16, 2025, which will cover: De-escalation strategies and trauma-informed debriefing, The requirement to complete a staff and resident debrief within 24 hours of an intervention, The procedure for repeated refusals and guardian notification. Attestations will be signed by all staff to verify completion and understanding of the training. Staff will not be allowed back on the floor if training is not completed by the due date and training must be completed before returning to work their shift. Monitoring: The Safety Team, in collaboration with the Director of Nursing and Director of Quality and Risk Management, will conduct audits (within one business day) of restraint episodes to confirm: Completion and documentation of staff and resident debriefs, Re-attempts for residents who initially refuse, Guardian notification when applicable. Audit findings will be reviewed monthly with the Executive Leadership Team during the Committee of Whole, and non-compliance will be addressed with retraining or corrective action. Monitoring will be ongoing until 100% compliance for three consecutive months. Following, monitoring/audits will occur bi-monthly. Completion Date (X5): August 16, 2025		

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N 189	Continued From page 9 On 07/09/2025 at 9:04 AM, during an interview, the Director of Nursing confirmed that copies of all emergency safety interventions were in Resident #2's chart and did not reveal any debriefing documentation for 07/07/25. Resident #5 Review of Resident #5 ' s Master Treatment Plan dated 05/23/25, indicated Resident #5 had diagnoses which included major depressive disorder, generalized anxiety disorder, post-traumatic stress disorder, and attention-deficit hyperactivity disorder. Review of an Emergency Intervention packet, completed and signed by the nurse on 05/23/25 at 12:43 PM, and signed by the physician on 05/23/25 at 12:35 PM, indicated Resident #5 was in a chemical restraint. The Staff Emergency Safety Intervention Debriefing form indicated that the date and time of debriefing was 05/23/25 at 12:40 PM. However, the chemical restraint ended at 12:43 PM. There was no documented staff debriefing after the restraint had ended. The policy titled, " Emergency Safety Interventions, " dated 01/2025, read in part, " within 24 hours from the initiation of the restraint, all staff involved in the ESI will meet in a post-intervention conference to discuss the event and offer feedback to one another concerning events that took place and possible alternate methods which could be used to change behaviors. "	N 189			



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& Quality Assurance
P.O. Box 8059, Slot S404
Little Rock, AR 72203-8059

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Charlotte Lockhart, Administrator
Woodridge Of Forrest City, Llc
1521 Albert St
Forrest City, AR 72335

Dear Ms. Lockhart:

On **July 14, 2025**, we conducted a **Complaint Investigation Survey** at your facility. You have alleged that the deficiencies cited on that survey have been corrected. We are accepting your allegation of compliance and have approved your plan of correction and presume that you will achieve substantial correction by **August 16, 2025**.

We will be conducting a revisit of your facility to verify that substantial correction has been achieved and maintained.

If you have any questions, please contact your reviewer: **Breanna Marengo at 501-320-6280 or email to: breanna.marengo@dhs.arkansas.gov**.

Sincerely,

A handwritten signature in black ink, appearing to read "B. Marengo". The signature is written in a cursive, flowing style.

for Jeff Rosenbaum

Breanna Marengo, Reviewer
DPSQA/Office of Long Term Care
Survey & Certification Section

bbm